

Meadowbrook Community Hospital

DISCHARGE SUMMARY

Patient: Maria Delgado

DOB: 02/09/1972 (Age 54)

MRN: MRN-2025-0187

Admission Date: 08/02/2026

Discharge Date: 08/05/2026

Attending Physician: Dr. Kevin Alabi, MD

ADMISSION DIAGNOSIS:

1. Community-acquired pneumonia, right lower lobe (J18.9)
2. Small parapneumonic effusion
3. Hypertension

HOSPITAL COURSE:

54-year-old female admitted with 4 days of productive cough, fever, and right-sided pleuritic pain. Imaging showed RLL consolidation with small parapneumonic effusion. Laboratory evaluation demonstrated leukocytosis with markedly elevated CRP and procalcitonin. Treated with IV ceftriaxone and azithromycin, transitioned to oral levofloxacin after becoming afebrile. Required up to 3L nasal cannula initially, weaned to room air by hospital day 3. Effusion remained small and stable; no drainage indicated. Blood cultures remained negative.

CONDITION AT DISCHARGE:

Afebrile x 36 hours, SpO2 96% on room air, ambulating without dyspnea at rest.

DISCHARGE MEDICATIONS:

1. Levofloxacin 750 mg PO daily x 5 more days (complete total course)
2. Lisinopril 10 mg PO daily (resumed)
3. Acetaminophen 650 mg PO q6h PRN pain

ALLERGIES: No known drug allergies

FOLLOW-UP:

- Primary care in 7 days with repeat chest x-ray to document resolution
- Repeat CRP not required if clinically improving
- Pneumococcal vaccination discussed - administer at PCP visit
- Return immediately for: fever recurrence, worsening dyspnea, hemoptysis

DISPOSITION:

Discharged to home. Activity as tolerated.